# M20 Hydrocortisone

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| **Scope of Practice** | EMT **IM** Administration<br>EMT-A **IV** Administration |
| **Classification** | • Corticosteroid |
| **Usual Presentation** | • 100mg/2ml (act-o-vial) |
| **Mechanism of Action** | • Hydrocortisone has anti-inflammatory and immunosuppressive effects.<br>• It reduces the production of inflammatory substances namely prostaglandins and leukotrienes, leading to a decrease in inflammation and immune activity |
| **Indications** | • Asthma<br>• COPD<br>• Anaphylaxis |
| **Contraindications** | • Known allergy |
| **Precautions** | • Concurrent fungal infections |
| **Side Effects** | • Fatigue<br>• Hypertension |
| **Adult Dose & Administration** | **Asthma**<br>• Moderate Asthma:<br>  • 100mg IV/IM<br>• Severe or life-threatening Asthma:<br>  • 200mg IV/IM/IO<br>**Anaphylaxis**<br>• 100-200mg IV/IM/IO<br>**COPD**<br>  • 100-200mg IM/IV/IO<br>• IV doses should be given slowly over 2 minutes or diluted in 100ml normal saline and administered over 2-5 minutes. |
| **Pediatric Dose & Administration** | **Asthma & Anaphylaxis**<br>• Moderate<br>  • IV/IM/IO 2mg/kg.<br>  • Or, for ease of dilution and administration:<br>  • **<1year:** 25mg<br>  • **1-5 years:** 50mg<br>  • **>5 years:** 100mg<br>• Severe<br>  • IV/IM/IO 4mg/kg |
| **Use in Pregnancy and Lactation** | • Pregnancy: Fetal harm has been demonstrated. Hydrocortisone should only be administered to severe or life-threatening cases.<br>• Lactation: Safety has not been established. If essential to be administered the mother should be advised to stop breastfeeding until advised by their physician. |
| **Additional Information** | • In severe asthma the dose may be increased to Adults: 200mg and Pediatrics:4mg/kg and should be given IV |